Prior Authorization Request Packet

Case pa_a055ce0e · Aetna · MRI knee (lower extremity joint) without contrast (73721) · Policy Aetna CPB 0171
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1. Letter of medical necessity

October 11, 2026

Aetna Utilization Management / Prior Authorization

RE: Marcus Lindqvist, DOB 06/23/1991, Member ID AET-513980027. Requested service: MRI knee (lower extremity joint) without contrast, CPT 73721

To the Medical Director:

I am writing on behalf of Dr. Priya Raman (NPI 1891234567) to request authorization for a right knee MRI without contrast. This request is reviewed against Aetna Clinical Policy Bulletin 0171 (https://www.aetna.com/cpb/medical/data/100_199/0171.html).

Clinical summary: Mr. Lindqvist is a "35-year-old male recreational soccer player" with "persistent right knee pain and intermittent swelling." The note records: "He recalls a twisting injury during a game earlier this summer." and "Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs." Examination documents "Moderate right knee effusion.", "Medial joint line tenderness.", "McMurray test positive medially with a palpable click.", "Lachman negative, posterior drawer negative." and "Range of motion 5 to 120 degrees, limited by pain." The assessment is "Suspected right medial meniscus tear with persistent effusion and instability."

Criteria review under CPB 0171:

Criterion 1, persistent knee pain, swelling, and/or instability (giving way): Met. The note states: "Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs."

Criterion 2, no response to at least 3 weeks of conservative therapy: Met. The clinic note states that "He has been doing physical therapy and taking ibuprofen, and has been using a brace" but gives no durations. The duration comes from a staff-entered answer in the record, not from the physician's note: "Yes. The patient completed about eight weeks of formal physical therapy, twice weekly, from early July through the end of August, plus daily NSAIDs." This is about eight weeks against the three required. The symptoms and effusion remain documented as persistent at the visit.

Criterion 3, multi-view x-rays ruling out fracture or loose body: Met. The note states: "Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint spaces preserved, small effusion."

The policy also lists indications that are not the basis of this request. For persistent true locking of the knee, the note says: "He denies true locking of the knee." This indication is not met and is not claimed. For suspected osteomyelitis or septic joint, the note records "No fever, no redness." and "No warmth or erythema." and no such concern is documented. No evidence of tumor of the knee is documented. The request rests on the first three criteria above.

Conclusion: The documentation shows persistent pain, swelling, and giving way after a twisting injury, and the examination supports a suspected medial meniscus tear. Multi-view radiographs have excluded fracture and loose body. Conservative therapy of about eight weeks is documented by staff entry. On this basis, the request meets the criteria of CPB 0171 for MRI of the knee, and we ask that CPT 73721 be approved. Dr. Raman can be reached for any additional information or peer-to-peer discussion.

Sincerely,

Priya Raman, MD
NPI 1891234567
Ordering Physician

2. Criteria checklist with verified evidence

MET aetna-73721-c1: Persistent knee pain, swelling, and/or instability (giving way).
"Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs."

MET aetna-73721-c2: Not responding to at least 3 weeks of conservative therapy (conservative therapy consists of a combination of rest, ice, compression, elevation, plus NSAIDs and/or physical therapy).
"Yes. The patient completed about eight weeks of formal physical therapy, twice weekly, from early July through the end of August, plus daily NSAIDs." [manual]
code check: 8.0 weeks documented vs 3 required

MET aetna-73721-c3: Multi-view x-rays have been obtained and have ruled out a fracture or loose body in the knee.
"Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint spaces preserved, small effusion."

MISSING aetna-73721-r1: Persistent true locking of the knee indicative of a torn meniscus or loose body (more than a momentary locking of the joint with the knee in a flexed position).

MISSING aetna-73721-r2: Suspected bone infection (i.e., osteomyelitis) or septic joint.

MISSING aetna-73721-r3: Detection, staging, and post-treatment evaluation of tumor of the knee.

3. Source visit note

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CHIEF COMPLAINT: Right knee pain and swelling.

HISTORY OF PRESENT ILLNESS: 35-year-old male recreational soccer player presents with
persistent right knee pain and intermittent swelling. He recalls a twisting injury during a
game earlier this summer. Since then he has had medial knee pain, swelling after activity,
and a sensation of the knee giving way on stairs. He denies true locking of the knee. No
fever, no redness. He has been doing physical therapy and taking ibuprofen, and has been
using a brace, but the symptoms have persisted.

PHYSICAL EXAM: Moderate right knee effusion. Medial joint line tenderness. McMurray test
positive medially with a palpable click. Lachman negative, posterior drawer negative. Range
of motion 5 to 120 degrees, limited by pain. No warmth or erythema.

IMAGING: Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint
spaces preserved, small effusion.

ASSESSMENT: Suspected right medial meniscus tear with persistent effusion and instability.

PLAN: MRI right knee without contrast (CPT 73721) to evaluate meniscus and ligaments.
Continue physical therapy pending imaging.
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